TATA 1mg | Labs®

# PERSONAL HEALTH SMART REPORT

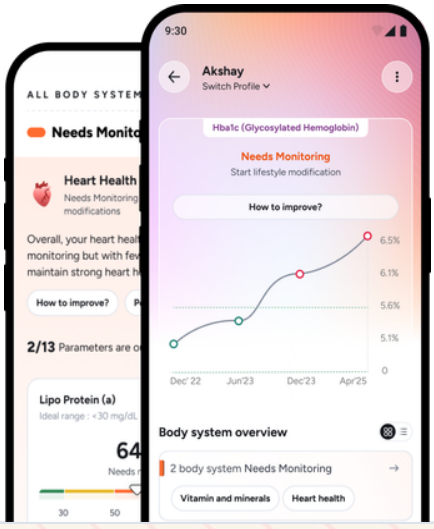
Prepared for
**Kislay**

Basic Info
**Male /20 Yrs**

Sample collection date
**12/10/2026**

View detailed health insights, and trends on Tata 1mg app.

Click here



TATA 1mg Labs®

| Patient ID | Date of Collection | Prepared for | Gender/age |
|---|---|---|---|
| OKH356838 | 12/10/26 | Kislay | Male /20 Yrs |

# Doctor Summary

FeverPackageAdvanced(Includes Dengue, Malaria & Typhoid Tests)

*Note* This is an electronically generated **summary of clinically significant parameters of the attached report.** It is advised to read this summary in conjunction with the attached report and to correlate it clinically. For the trends section, the out-of-range values are highlighted with respect to the bio-reference range of respective reports.

| Test Name | Result | Bio. Ref. Interval |
|---|---|---|
| Complete Blood Count | 12 Oct 26 | |
| Hemoglobin | 15.9 g/dL | 13.0 - 17.0 |
| RBC | 5.15 10^6/cu.mm | 4.5 - 5.5 |
| HCT | 45.8 % | 40 - 50 |
| **MCHC** | **34.6** g/dL | 31.5 - 34.5 |
| **RDW-CV** | **14.1** % | 11.5 - 14 |
| **Total Leucocyte Count** | **3.36** 10^3/µL | 4 - 10 |
| Neutrophils | 40 % | 40 - 80 |
| **Lymphocytes** | **50** % | 20 - 40 |
| Monocytes | 08 % | 2 - 10 |
| Eosinophils | 02 % | 1 - 6 |
| Basophils | 00 % | 0 - 2 |
| **Absolute Neutrophil Count** | **1.34** 10^3/µL | 2 - 7 |
| **Absolute Basophil Count** | **0** 10^3/µL | 0.02 - 0.1 |
| **Platelet Count** | **96** 10^3/µL | 150 - 410 |
| **PDW** | **25.2** fL | 9 - 17 |

**Trends** (For last three tests)



Book more tests with Tata 1mg to start viewing trends. Your health journey starts here!

# Doctor Summary

FeverPackageAdvanced(Includes Dengue, Malaria & Typhoid Tests)

| Test Name | Result | Bio. Ref. Interval |
|---|---|---|
| **Inflammatory markers** | **12 Oct 26** | |
| Erythrocyte Sedimentation Rate | 7 mm/hr | 0.5 - 10 |
| **C-Reactive Protein (Quantitative)** | **20.70** mg/L | 0 - 3.3 |
| **Liver Health** | **12 Oct 26** | |
| **Aspartate Transaminase (SGOT)** | **70** U/L | 0 - 33.9 |
| Alanine Transaminase (SGPT) | 31 U/L | 10 - 49 |
| **Urine Routine & Microscopy** | **12 Oct 26** | |
| **Colour** | **Amber** | Pale yellow |
| Specific gravity | 1.026 | 1.000 - 1.030 |
| pH | 5.5 | 5 - 8 |
| Glucose | Negativ e | Negative |
| Protein | Negativ e | Negative |
| **Ketones** | **Trace** | Negative |
| Pus cells | 1.40 /hpf | 0 - 5 |
| Red blood cell | 0.20 /hpf | 0 - 2 |

**Trends** (For last three tests)



Book more tests with Tata 1mg to start viewing trends. Your health journey starts here!

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Patient ID **OKH356838** | Date of Collection **12/10/26** | Prepared for **Kislay** | Gender/age **Male /21 Yrs**

# Doctor Summary

FeverPackageAdvanced(Includes Dengue, Malaria & Typhoid Tests)

| Test Name | Result | Bio. Ref. Interval | **Trends** (For last three tests) |
|---|---|---|---|
| Dengue Fever NS1 Antigen | 12 Oct 26 | | |
| **Result** | **3.4** Index | <= 0.9 | |
| Dengue Panel | 12 Oct 26 | | |
| **Dengue NS1** | **Positive** | Negative | |
| Infection profile | 12 Oct 26 | | |
| **Salmonella typhi (Typhidot) IgM** | **Positive** | Negative | |



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PO No :PO10003864883-984

| | | | |
|---|---|---|---|
| Customer Name | : Mr.Kislay | Collected Via | : TATA 1MG OKHLA |
| Age/Gender | : 20Y 8M 29D /Male | Referred By | : Self |
| Lab Visit ID | : OKH3356838 | Collection Date | : 12/Oct/2026 12:21PM |
| Barcode ID/Order ID | : D3504696 / 1854653 : | Report Date | : 12/Oct/2026 10:26PM |
| Sample Type | :WHOLE BLOOD-EDTA | Report Status | : Final Report |

## *HAEMATOLOGY*

### *FEVER PACKAGE ADVANCED*

| *Test Name* | *Result* | *Unit* | *Bio. Ref. Interval* | *Method* |
|---|---|---|---|---|
| ***Complete Blood Count*** | | | | |
| Hemoglobin | 15.9 | g/dL | 13.0-17.0 | Spectrophotometry (Cyanide-free) |
| RBC | 5.15 | 10^6/cu.mm | 4.5 - 5.5 | Impedance |
| HCT | 45.8 | % | 40 - 50 | Calculated |
| MCV | 88.9 | fL | 83 - 101 | Calculated |
| MCH | 30.8 | pg | 27 - 32 | Calculated |
| MCHC | ***34.6*** | g/dL | 31.5 - 34.5 | Calculated |
| RDW-CV Total Leucocyte | ***14.1*** | % | 11.5-14 | Calculated |
| Count ***Differential Leucocyte*** | ***3.36*** | 10^3/µL | 4 - 10 | Impedance |
| ***Count*** Neutrophils | | | | |
| Lymphocytes Monocytes | 40 | % | 40-80 | DHSS/Microscopy |
| Eosinophils Basophils | ***50*** | % | 20-40 | DHSS/Microscopy |
| ***Absolute Leucocyte Count*** | 08 | % | 2-10 | DHSS/Microscopy |
| Absolute Neutrophil Count | 02 | % | 1-6 0- | DHSS/Microscopy |
| Absolute Lymphocyte Count | 00 | % | 2 | Impedance/Microscopy |
| Absolute Monocyte Count | | | | |
| Absolute Eosinophil Count | ***1.34*** | 10^3/µL | 2 - 7 | Calculated |
| Absolute Basophil Count | 1.68 | 10^3/µL | 1-3 | Calculated |
| Platelet Count | 0.27 | 10^3/µL | 0.2 - 1 | Calculated |
| MPV | 0.07 | 10^3/µL | 0.02 - 0.5 | Calculated |
| PDW | ***0*** | 10^3/µL | 0.02-0.1 | Calculated |
| | ***96*** | 10^3/µL | 150-410 | Impedance/Microscopy |
| | 11.4 | fL | 6.5 - 12 | Calculated |
| | ***25.2*** | fL | 9-17 | Calculated |

PLATELET COUNT rechecked at our end, suggest repeat test,if value do not correlate with clinical picture.
Platelet counts verified on the smear.

***Comment:***
Asperthe recommendation of International council for Standardization in Hematology, the differential leucocyte counts are additionally being reported as absolute numbers of each cell in per unit volume of blood.

NABL certificate and scope

MC - 5424

This test has been performed at
***TATA 1MG OKHLA***
2nd Floor, B-225, Okhla Phase I, Okhla Industrial Estate, New Delhi, Delhi 110020

Dr. Dhananjay Singh
MBBS, MD(Pathology)
Consultant Pathologist
Reg No: 63325

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## *HAEMATOLOGY*

### *FEVER PACKAGE ADVANCED*

| *Test Name* | *Result* | *Unit* | *Bio. Ref. Interval* | *Method* |
|---|---|---|---|---|

DHSS : Double Hydrodynamic Sequential System Flowcytometry
Calculated parameters are either derived from Impedence measure, RBC pulse measurement, RBC/platelet histograms or formula derived.

### *Erythrocyte Sedimentation Rate*

| | | | | |
|---|---|---|---|---|
| Erythrocyte Sedimentation Rate | 7 | mm/hr | 0-10 | Capillary Photometry |

***Comment:***

- ESR provides an index of progress of the disease and is widely used as an indicator of inflammation, infection, trauma, or malignant diseases. Changes are more significant than a single abnormal test
- It is specifically indicated to monitor the course or response to the treatment of diseases like rheumatoid arthritis, tuberculosis bacterial endocarditis ,acute rheumatic fever ,Hodgkins disease,temporal arthritis , and systemic lupus erythematosis; and to diagnose and monitor giant cell arteritis and polymyalgia rheumatica.
- An elevated ESR may also be associated with many other conditions, including autoimmune disease, anemia, infection,malignancy,pregnancy, multiple myeloma, menstruation, and hypothyroidism.
- Although a normal ESR cannot be taken to exclude the presence of organic disease, its rate is dependent on various physiologic and pathologic factors.
- The most important component influencing ESR is the composition of plasma. High level of C-Reactive Protein, fibrinogen, haptoglobin, alpha-1antitrypsin, ceruloplasmin and immunoglobulins causes the elevation of Erythrocyte Sedimentation Rate.
- Drugs that may cause increase ESR levels include: dextran, methyldopa, oral contraceptives, penicillamine, procainamide, theophylline, and Vitamin A. Drugs that may cause decrease levels include: aspirin, cortisone, and quinine



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## *HAEMATOLOGY*

### *FEVER PACKAGE ADVANCED*

| *Test Name* | *Result* | *Unit* | *Bio. Ref. Interval* | *Method* |
|---|---|---|---|---|
| ***Malarial Antigen (Vivax & Falciparum) Detection*** | | | | |
| Plasmodium falciparum Antigen | Negative | | Negative | Immunochromatography |
| Plasmodium vivax Antigen | Negative | | Negative | Immunochromatography |

***Comment:***

- Four species of the Plasmodium parasites are responsible for human malaria infection - P.falciparum, P.vivax, P.ovale and P.malariae. P.falciparum and P.vivax are the most prevalent . Falciparum infection is associated with Cerebral malaria and drug resistance whereas vivax infection is associated with high rate of infectivity and relapse. Differentiation between P.falciparum and P.vivax is of utmost importance for better patient management and speedy recovery.
- This is only a screening test. The results must always be correlated with clinical history and relevant epidemiological and therapeutic context.
- A Positive result indicates malarial infection. False Positives may be seen due to cross reactivity and persistence antigenemia.
- False negatives may be seen in patient's with very low parasitic index .

Malaria P.f/P.v Ag Test is an immunochromatographic assay for the differential detection between Plasmodium falciparum Histidine -Rich Protein-II (HRP-II) and pLDH (plasmodium lactate dehydrogenase) specific to Plasmodium vivax in human whole blood.

***Note:***

- HRP-2/pLDH antigens may persist for days to weeks after parasite clearance; therefore, antigen positivity does not necessarily indicate active infection and should not be used to assess treatment response. Smear examination is preferred for assessing active parasitemia.
- In very low parasitemia, antigen testing may be positive despite a negative smear examination.

NABL certificate and scope

MC - 5424

This test has been performed at
***TATA 1MG OKHLA***
2nd Floor, B-225, Okhla Phase I, Okhla Industrial Estate, New Delhi, Delhi 110020

Dr. Kundan Kumar
MBBS, MD ( Lab Medicine)
Consultant Laboratory Medicine
Reg No- 96030

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## *BIOCHEMISTRY*

### *FEVER PACKAGE ADVANCED*

| *Test Name* | *Result* | *Unit* | *Bio. Ref. Interval* | *Method* |
|---|---|---|---|---|
| ***SGOT (Aspartate Aminotransferase)*** | | | | |
| SGOT (Aspartate Aminotransferase) | ***70*** | U/L | <34 | Modified IFCC |

***Comment:***
SGOT/AST :

- Present in large concentrations in liver, skeletal muscle, brain, red cells, and heart.
- Released into the bloodstream when tissue is damaged, especially in liver injury.
- Test is not indicatted for diagnosis of myocardial infarction.
- AST/ALT ratio>1 suggests cirrhosis in patients wiyh hepatitis C.

Increased in: Acute viral hepatitis (ALT> AST),
: Biliary tract obstruction (cholangitis, choledocholithiasis),
: Alcoholic hepatitis and cirrhosis (AST> ALT)
: Other conditions - liver abscess, metastatic or primary liver cancer; right heart failure, ischemia or hypoxia, injury to liver("shock liver"), extensive trauma. Drugs that cause cholestasis or hepatotoxicity.

Decreased in: Pyridoxine (vitamin B6) deficiency.

| | | | | |
|---|---|---|---|---|
| ***SGPT (Alanine Transaminase)*** | | | | |
| SGPT (Alanine Transaminase) | 31 | U/L | 10-49 | Modified IFCC |

***Comment:***
***SGPT/ALT :***

- Present in large concentrations in liver, kidney; in smaller amounts, in skeletal muscle and heart.
- Released with tissue damage, particularly liver injury. ALT is the preferred enzyme for evaluation of liver injury.

***Increased in:*** Acute viral hepatitis (ALT> AST)
:Biliary tract obstruction (cholangitis, choledocholithiasis)
:Alcoholic hepatitis and cirrhosis (AST> ALT)
:Other conditions - liver abscess, metastatic or primary liver cancer;right heart failure, ischemia or hypoxia, injury to liver ("shock liver"),extensive trauma. Drugs that cause cholestasis or atotoxicity.
***Decreased in:*** Pyridoxine (vitamin B6) deficiency.

***C-Reactive Protein Quantitative***

NABL certificate and scope

MC - 5424

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## *BIOCHEMISTRY*

### *FEVER PACKAGE ADVANCED*

| *Test Name* | *Result* | *Unit* | *Bio. Ref. Interval* | *Method* |
|---|---|---|---|---|
| C-Reactive Protein (Quantitative) | ***20.70*** | mg/L | 0-3.3 | Immunoturbidimetric |

***Comment:***

- C-Reactive Protein [CRP] is an acute phase reactant ,hepatic secretion of which is stimulated in response to inflammatory cytokines.
- CRP is a very sensitive but nonspecific marker of inflammation and infection.
- The CRP test is useful in patient with Inflammatory bowel disease, arthritis, Autoimmune diseases, Pelvic inflammatory disease (PID), tissue injury or necrosis and infections.
- CRP levels can be elevated in the later stages of pregnancy as well as with use of birth control pills or hormone replacement therapy i.e. estrogen. Higher levels of CRP have also been observed in the obese.
- As compared to ESR, CRP shows an earlier rise in inflammatory disorders which begins in 4-6 hrs, he intensity of the rise being higher than ESR and the recovery being earlier than ESR. Unlike ESR, CRP levels are not influenced by hematologic conditions like Anemia, Polycythemia.

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NABL certificate and scope

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## *SEROLOGY*

### *FEVER PACKAGE ADVANCED*

| *Test Name* | *Result* | *Unit* | *Bio. Ref. Interval* | *Method* |
|---|---|---|---|---|
| ***DENGUE FEVER NS1 ANTIGEN*** | | | | |
| ***Dengue NS1 Antigen*** | | | | |
| Result | ***3.4*** | Index | Negative (<0.9)<br>Equivocal (0.9- <1.1)<br>Positive (>1.1) | CLIA |
| Dengue NS1 | ***Positive*** | | Negative | |

***Critical Result-(Dengue NS1 ) . Please consult your doctor immediately.***
***Advice -Kindly correlate clinically.Repeat testing may be done on a fresh sample, if clinically indicated.***

***Comment:***
Note:Change in method and reference interval

- Dengue NS1 is a qualitative assay. The reported index (S/CO) value indicates reactivity only; its magnitude does not correlate with viral load, disease pathogenesis, severity, prognosis, or clinical improvement. Results should be interpreted with the clinical findings and other relevant laboratory parameters.
- Recommended test is NS1 Antigen by ELISA/CLIA in the first 5 days of fever. After 7-10 days of fever, the recommended test is Dengue virus antibodies IgG & IgM by ELISA/CLIA
- Dengue viruses belong to the family Flaviviridae and have 4 serotypes (1 - 4). It is transmitted by the mosquito Aedes aegypti and Aedes albopictus and is widely distributed in Tropical and Subtropical areas of the world.
- The disease may be subclinical, self limiting, febrile or may progress to a severe form of Dengue hemorrhagic fever or Dengue shock syndrome.
- Positive: The presence of Dengue Nonstructural protein 1 (NS1) antigen is typically detectable within 1 to 2 days following infection and may persist up to 9 days following symptom onset. NS1 antigen may also be detectable during secondary dengue virus infection, but for a shorter duration of time (1 - 4 days following symptom onset).
- Negative: The absence of Dengue NS1 antigen suggests that the infection may not be in the acute phase. NS1 might be negative if the test is done too early (within 24-48 hours of infection) or too late (9-10 days after symptoms start). In such cases, serologic testing for IgM and IgG antibodies is recommended.
- Results should always be interpreted in conjunction with clinical presentation and exposure history.
-

Limitations:

- False positive results can be obtained due to cross reaction with Murray Valley and encephalitis, Japanese encephalitis, yellow fever and West Nile viruses. This occurs in less then 1% of the sample tested.

Note: The referring centre/ Lab is responsible for informing concerned Local authorities on notifiable disease.

NABL certificate and scope

MC - 5424

This test has been performed at
***TATA 1MG OKHLA***
2nd Floor, B-225, Okhla Phase I, Okhla Industrial Estate, New Delhi, Delhi 110020

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## *SEROLOGY*

### *FEVER PACKAGE ADVANCED*

| *Test Name* | *Result* | *Unit* | *Bio. Ref. Interval* | *Method* |
|---|---|---|---|---|

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## *SEROLOGY*

### *FEVER PACKAGE ADVANCED*

| *Test Name* | *Result* | *Unit* | *Bio. Ref. Interval* | *Method* |
|---|---|---|---|---|
| ***Salmonella typhi (Typhidot) IgG & IgM*** | | | | |
| Salmonella typhi (Typhidot) IgM | ***Positive*** | | NEGATIVE | Immunochromatography |
| Salmonella typhi (Typhidot) IgG | Negative | | NEGATIVE | Immunochromatography |

***Comment:***
Typhoid fever is an infection caused by a bacterium, Salmonella Typhi. Timely diagnosis of typhoid fever at an early stage is not only important for etiological diagnosis but also to identify and treat the potential carrier state in order to prevent acute typhoid fever outbreaks. TYPHIDOT is an immunochromatographic assay designed for the qualitative detection and differentiation of specific IgM and IgG antibodies against specific Salmonella Typhi antigen in human serum or plasma. This test is an aid in the early diagnosis of typhoid infection.

Note:-

- It is a rapid, qualitative, screening test for early detection of antibodies to Salmonella Typhi in human serum/plasma. All positive results should be confirmed by supplement tests.
- A negative result does not rule out recent infection, as positive result is influenced by the time elapsed after the onset of fever and immuno- competence of the patient.

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## *SEROLOGY*

## *FEVER PACKAGE ADVANCED*

### *Widal Test (Slide Agglutination)*

Result

| S.No. | ANTIGEN | OBSERVED TITRE | METHOD |
|---|---|---|---|
| 1 | Salmonella Typhi 'O' | <1:20 | Slide agglutination |
| 2 | Salmonella Typhi 'H' | <1:20 | Slide agglutination |
| 3 | Salmonella Paratyphi 'AH' | <1:20 | Slide agglutination |
| 4 | Salmonella Paratyphi 'BH' | <1:20 | Slide agglutination |

Interpretation:
• This is a screening test for antibodies against Salmonella typhi and Salmonella paratyphi A & B. Results indicate only approximate titres.
• Titers ≥ 1:80 of O antigen and ≥ 1:160 of H antigen of S. typhi or S. paratyphi A & B are considered significant.
• A single titre has limited diagnostic value; paired samples taken 7-10 days apart are more significant.
• A positive Widal test indicates Enteric fever or typhoid fever and should be confirmed by gold standard test such as Blood culture.

Limitation:
• False Positive: Anamnestic response may be seen in persons with prior enteric infection or immunization with TAB vaccination. False positive results may be seen during unrelated fevers like Malaria, Tuberculosis, Dengue, Brucellosis, Rheumatic fever etc. In such cases, there is a temporary increase in 'H' antibody titer, whereas enteric fever patients show a sustained rise in both 'H' and 'O' antibodies
• In endemic areas, normal people may show moderately elevated levels of 'O' and 'H' agglutinins
• False negative: Agglutinins usually appear by the end of the first week of infection, blood sample taken earlier may give a negative result. Immunosuppression or antibiotic treatment during the first week, before antibodies develop, may suppress the immune response, leading to lower antibody levels.

Comments:
• The Widal test is a serological assay used to detect antibodies against Salmonella antigens ('H' and 'O') in the serum and help to detect infections due to S. typhi or S. paratyphi A & B.
• Results should always be correlated with clinical findings to arrive at the final diagnosis.
• The Widal Slide agglutination test can occasionally produce false positive results; confirmation may be done by the Tube Widal test.

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## *CLINICAL PATHOLOGY*

### *FEVER PACKAGE ADVANCED*

| *Test Name* | *Result* | *Unit* | *Bio. Ref. Interval* | *Method* |
|---|---|---|---|---|
| ***Urine Routine & Microscopy*** | | | | |
| ***Urine Routine*** | Automated | | | |
| ***Physical & Chemical Examination*** | | | | |
| Colour | ***Amber*** | | Pale Yellow | Optical array |
| Appearance | Clear | | Clear | Optical array |
| Specific gravity | 1.026 | | 1.000–1.030 | Refractometry |
| pH | 5.5 | | 5.0 - 8.0 | Double Indicator |
| Glucose | Negative | | Negative | Glucose-oxidase |
| Protein | Negative | | Negative | Tetra-bromphenolblue |
| Ketones | ***Trace*** | | Negative | Sodium Nitroprusside |
| Rechecked with manual method. | | | | |
| Blood | Negative | | Negative | Tetramethyl benzidine |
| Bilirubin | Negative | | Negative | Diazonium salt |
| Urobilinogen | Normal | | Normal | Diazonium salt |
| Leucocyte Esterase | Negative | | Negative | Carboxylic acid ester |
| Nitrite | Negative | | Negative | Griess reaction |
| Ascorbic acid | Negative | | Negative | Indophenol |
| ***Microscopic Examination*** | | | | |
| Pus cells | 1.40 | /hpf | 0.0-5.0 | |
| Red Blood Cells | 0.20 | /hpf | 0.0-2.0 | |
| Epithelial cells | 0.00 | /hpf | 0.0-5.0 | |
| Yeast | 0.00 | /hpf | 0.0-1.0 | |
| Bacteria | Nil | /hpf | 0.0-80.0 | |
| Hyaline casts | 0.00 | /hpf | 0.0-5.0 | |
| RBC casts | 0.00 | /hpf | 0.0-0.99 | |
| WBC casts | 0.00 | /hpf | 0.0-0.99 | |
| Hyaline-granular casts | 0.00 | /hpf | 0.0-0.99 | |
| Fatty casts | 0.00 | /hpf | 0.0-0.99 | |
| Granular casts | 0.00 | /hpf | 0.0-0.99 | |
| Waxy casts | 0.00 | /hpf | 0.0-0.99 | |

NABL certificate and scope

This test has been performed at
***TATA 1MG OKHLA***
2nd Floor, B-225, Okhla Phase I, Okhla Industrial Estate, New Delhi, Delhi 110020

Ranjan yadav
**Dr. Ranjan Yadav**
MBBS, MD (Laboratory Medicine),
Consultant Physician, Laboratory Medicine,
Reg No: 91820,

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| | | | |
|---|---|---|---|
| Customer Name | : Mr.Kislay | Collected Via | : TATA 1MG OKHLA |
| Age/Gender | : 20Y 8M 29D /Male | Referred By | : Self |
| Lab Visit ID | : OKH3356838 | Collection Date | : 12/Oct/2026 12:21PM |
| Barcode ID/Order ID | : D3504696 / 1854653 : | Report Date | : 12/Oct/2026 10:26PM |
| Sample Type | :WHOLE BLOOD-EDTA | Report Status | : Final Report |

## *CLINICAL PATHOLOGY*

### *FEVER PACKAGE ADVANCED*

| *Test Name* | *Result* | *Unit* | *Bio. Ref. Interval* | *Method* |
|---|---|---|---|---|
| Microorganism casts | 0.00 | /hpf | 0.0-0.99 | |
| Calcium oxalate monohydrate crystals | 0.10 | /hpf | 0.0-0.99 | |
| Calcium oxalate dihydrate crystals | 0.00 | /hpf | 0.0-0.99 | |
| Triple phosphate crystals | 0.00 | /hpf | 0.0-0.99 | |
| Uric acid crystals | 0.00 | /hpf | 0.0-0.99 | |
| Calcium phosphate | 0.00 | /hpf | 0.0-0.99 | |
| Cystine crystals | 0.00 | /hpf | 0.0-0.99 | |
| Leucine crystals | 0.00 | /hpf | 0.0-0.99 | |
| Tyrosine crystals | 0.00 | /hpf | 0.0-0.99 | |
| Amorphous Material | 0.00 | /hpf | 0.0-0.99 | |

***Comment:***

- Urine microscopy is done in centrifuged urine specimens.
- Results should be interpreted in conjunction with clinical findings and other laboratory investigations.
- For optimum results, the sample should be a first-void, mid-stream urine collected in a clean, dry, and sterile container. Contamination from vagina, urethra, perineum may affect results.
- Presence of red blood cells may be influenced by menstruation.
- Certain medications, dehydration, or supplements may impact results.
- Microscopy findings are semi-quantitative and do not confirm infection; urine culture may be required.
- Trace proteinuria may be observed in various physiological conditions such as prolonged recumbency, strenuous exercise, or a high-protein diet.
- Interference: False-positive results for bile pigments, protein, glucose, and nitrites may occur due to disinfectants, peroxide-based cleaning agents, therapeutic dyes, vitamin C, or certain medications.
- Ascorbic acid (Vitamin C): May interfere with glucose, nitrite, bilirubin, and blood results; repeat testing is recommended after at least 10 hours or next day after stopping vitamin C.
- Blood: Small amounts of blood may not change urine colour and are detectable only by tests; false positives may occur due to infection or contamination. Occasionally strip tests may show higher value than microscopy due to lysed RBC.
- Leukocyte esterase: Results may be influenced by drugs, coloured compounds, high glucose or protein, and urine concentration; false positives can occur due to vaginal contamination or preservatives. Occasionally strip values may exceed microscopy due to lysed white blood cells or pus cells.

*** End Of Report ***

Disclaimer:
1. The reported results based on laboratory investigation, are only for the purposes of diagnosis and should be clinically correlated and interpreted

NABL certificate and scope

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## *CLINICAL PATHOLOGY*

### *FEVER PACKAGE ADVANCED*

| *Test Name* | *Result* | *Unit* | *Bio. Ref. Interval* | *Method* |
|---|---|---|---|---|

by the referring physician/ medical practitioner. For any queries relating to the reported results, you may write to our customer support team on care@1mg.com
2. It is presumed that the tests performed are, on the specimen / sample being to the patient named or identified and the verifications of particulars have been confirmed by the patient or his / her representative at the point of generation of said specimen.
3. The reported results are restricted to the given specimen only. Results may vary from lab to lab and from time to time for the same parameter for the same patient (within subject biological variation).
4. The patient's details along with their results in certain cases like notifiable diseases and as per local regulatory requirements will be communicated to the assigned regulatory bodies.
5. The patient samples can be used as part of internal quality control, test verification, data analysis purposes within the testing scope of the laboratory.
6. This report is not valid for medico legal purposes. It is performed to facilitate medical diagnosis only.
7. Pregnant women should seek guidance from a qualified obstetrician as test parameters may vary during pregnancy

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NABL certificate and scope

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